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Dr Penny Clifton
Thoracic Surgeon
Department of Cardiothoracic Surgery
Central Hospital
Main Street
Stillwater

22/08/2015

Dear Dr Clifton,

Re: Mrs Mary Clarke

DOB: 17/09/1960

I am writing to refer Mrs Clarke, whose symptoms and signs are suggestive of bronchogenic carcinoma, for further investigations and assessment.

On today's visit, Mrs Clarke presented with a 7-week history of non-productive cough. It was preceded by flu-like symptoms which improved after Augmentin had been taken. On further discussion, mild dyspnea (which got worse at night) associated with a chest heaviness sensation were reported. Despite the previously mentioned, her daily life activities are not severely affected. Haemoptysis, along with fever and night sweats or rigors, were denied.

Apart from signs of consolidation coincided with a monophonic wheeze in the middle zone of the right lung, no other abnormalities were elicited upon her respiratory examination. She has no apparent signs of cyanosis, dyspnea or ascites. In addition, no cervical lymphadenopathy, hepatosplenomegaly, or bone pain were detected. Hoarseness of voice along with Horner's syndrome were not noted. For this reason, sputum cytology and a chest x-ray and CT were requested. They revealed normal findings and a middle lobe atelectasis with an enlarged hilum in the right lung, respectively. Mrs Clarke

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Mrs Clarke, who is a smoker of 30 to 35 cigarettes per day, has a maternal history of laryngeal carcinoma. She has not been receiving any long-term medications and has no known allergies. She presented 6 weeks earlier with a sore throat and body aches associated with fever and cough for which she had been prescribed Augmentin.

Consequently, Mrs Clarke was counselled on her suggested diagnosis. Kindly arrange for a bronchoscopy and a biopsy if needed.

Should there be any queries, please contact me.

Yours sincerely,

Doctor